BASELINE → TREATMENT → FOLLOW-UP

Your tests and treatment checks

Safety and response tests have different purposes. Use the individual prescription and appointment dates.

Check	Before starting	First treatment	Between appointments	Next review	Longer-term care
Blood and organ safety	Full blood count, kidney and liver tests, blood glucose and a bone blood profile. Review heart disease, blood pressure, diabetes, mood, falls and fracture risk, sexual health, medicines and the treatment goal.	Confirm baseline results and the starting plan.	Repeat blood count, kidney and liver tests, glucose and bone blood profile when clinically indicated. PSA and testosterone assess response. A suspected preparation or administration problem needs review and a testosterone check.	Confirm results and symptoms before the next prescription or injection.	Review bone, metabolic and heart health.
Treatment and appointments	Agree treatment aim, formulation, duration and medicines list.	A trained healthcare professional prepares and gives this specific formulation under the skin. Correct mixing and administration matter for hormone suppression. Different leuprorelin products and strengths have different instructions; they must not be substituted by matching the number of milligrams alone.	Keep an accurate medicine / injection record.	Confirm the exact next injection date. With radiotherapy, ask when hormone treatment starts and ends and how the radiotherapy dates fit. A low PSA does not mean you should stop injections. Tell the team about worsening pain, trouble passing urine or severe headache with vision changes.	Continue until the team changes the plan.
Cancer response	Prostate-specific antigen (PSA) helps assess cancer activity. Testosterone checks the effect of hormone suppression, especially if response is uncertain. Your team interprets these with symptoms and scans.	Document the baseline PSA and clinical picture.	Ask when the next PSA and any testosterone test are due.	Review symptoms and results together.	Scans follow the individual plan; there is no universal interval.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00479 v3: official regimen · Eligard: product information

Tests in your plan

Blood count, kidney and liver tests

WHEN

Repeat blood count, kidney and liver tests, glucose and bone blood profile when clinically indicated. PSA and testosterone assess response. A suspected preparation or administration problem needs review and a testosterone check.

Checks blood cells and organ health.

Blood glucose and bone blood profile

WHEN

Repeat blood count, kidney and liver tests, glucose and bone blood profile when clinically indicated. PSA and testosterone assess response. A suspected preparation or administration problem needs review and a testosterone check.

Checks metabolic effects and blood minerals.

PSA and testosterone

If part of your plan

WHEN

Prostate-specific antigen (PSA) helps assess cancer activity. Testosterone checks the effect of hormone suppression, especially if response is uncertain. Your team interprets these with symptoms and scans.

Checks cancer activity and hormone suppression.

electrocardiogram (ECG) and bone-density assessment

If part of your plan

WHEN

ECG (heart tracing) and bone-density scans are arranged according to heart-rhythm risk, fracture risk and clinical need. Your team confirms which tests you need.

Checks heart rhythm or fracture risk when indicated.

THE WHOLE TREATMENT JOURNEY

Leuprorelin 22.5 mg: hormone injection every 12 weeks: treatment and checks

Hormone-dependent advanced prostate cancer, or high-risk localised/locally advanced hormone-dependent disease in combination with radiotherapy.

Leuprorelin 22.5 mg is injected under the skin every 12 weeks (84 days) in this NCCP regimen. The prescribed duration depends on whether it supports radiotherapy or treats advanced disease.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First treatment	Between appointments	Next review	Longer-term care
Blood and organ safety NCCP · SMPC	Full blood count, kidney and liver tests, blood glucose and a bone blood profile. Review heart disease, blood pressure, diabetes, mood, falls and fracture risk, sexual health, medicines and the treatment goal.	Confirm baseline results and the starting plan.	Repeat blood count, kidney and liver tests, glucose and bone blood profile when clinically indicated. PSA and testosterone assess response. A suspected preparation or administration problem needs review and a testosterone check.	Confirm results and symptoms before the next prescription or injection.	Review bone, metabolic and heart health.
Treatment and appointments NCCP · SMPC	Agree treatment aim, formulation, duration and medicines list.	A trained healthcare professional prepares and gives this specific formulation under the skin. Correct mixing and administration matter for hormone suppression. Different leuprorelin products and strengths have different instructions; they must not be substituted by matching the number of milligrams alone.	Keep an accurate medicine / injection record.	Confirm the exact next injection date. With radiotherapy, ask when hormone treatment starts and ends and how the radiotherapy dates fit. A low PSA does not mean you should stop injections. Tell the team about worsening pain, trouble passing urine or severe headache with vision changes.	Continue until the team changes the plan.
Cancer response NCCP · SMPC	Prostate-specific antigen (PSA) helps assess cancer activity. Testosterone checks the effect of hormone suppression, especially if response is uncertain. Your team interprets these with symptoms and scans.	Document the baseline PSA and clinical picture.	Ask when the next PSA and any testosterone test are due.	Review symptoms and results together.	Scans follow the individual plan; there is no universal interval.

What each check is for

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Treatment and review

One injection every 12 weeks

This is androgen deprivation therapy (ADT): it lowers testosterone to help control prostate cancer. With radiotherapy, the aim may be cure; in advanced disease, it is part of ongoing cancer control.

Ask / act: Repeat blood count, kidney and liver tests, glucose and bone blood profile when clinically indicated. PSA and testosterone assess response. A suspected preparation or administration problem needs review and a testosterone check.

Sources: NCCP · SMPC

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-14 · Source reconciled draft; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.52